

Apheresis (Therapeutic Pheresis)

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Tracking Information

Publication Number

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Manual Section Number

110.14

Manual Section Title

Apheresis (Therapeutic Pheresis)

Version Number

1

Effective Date of this Version

07/30/1992

Description Information

Benefit Category

Incident to a physician's professional Service

Outpatient Hospital Services Incident to a Physician's Service

Physicians' Services

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description

A. General

Apheresis (also known as pheresis or therapeutic pheresis) is a medical procedure utilizing specialized equipment to remove selected blood constituents (plasma, leukocytes, platelets, or cells) from whole blood. The remainder is retransfused into the person from whom the blood was taken.

For purposes of Medicare coverage, apheresis is defined as an autologous procedure, i.e., blood is taken from the patient, processed, and returned to the patient as part of a continuous procedure (as distinguished from the procedure in which a patient donates blood preoperatively and is transfused with the donated blood at a later date).

Indications and Limitations of Coverage

B. Indications

Apheresis is covered for the following indications:

- Plasma exchange for acquired myasthenia gravis;
- Leukapheresis in the treatment of leukemia;
- Plasmapheresis in the treatment of primary macroglobulinemia (Waldenstrom);
- Treatment of hyperglobulinemias, including (but not limited to) multiple myelomas, cryoglobulinemia and hyperviscosity syndromes;
- Plasmapheresis or plasma exchange as a last resort treatment of thrombotic thrombocytopenic purpura (TTP);
- Plasmapheresis or plasma exchange in the last resort treatment of life threatening rheumatoid vasculitis;
- Plasma perfusion of charcoal filters for treatment of pruritis of cholestatic liver disease;
- Plasma exchange in the treatment of Goodpasture's Syndrome;
- Plasma exchange in the treatment of glomerulonephritis associated with antiglomerular basement membrane antibodies and advancing renal failure or pulmonary hemorrhage;
- Treatment of chronic relapsing polyneuropathy for patients with severe or life threatening symptoms who have failed to respond to conventional therapy;
- Treatment of life threatening scleroderma and polymyositis when the patient is unresponsive to conventional therapy;
- Treatment of Guillain-Barre Syndrome; and
- Treatment of last resort for life threatening systemic lupus erythematosus (SLE) when conventional therapy has failed to prevent clinical deterioration.

C. Settings

Apheresis is covered only when performed in a hospital setting (either inpatient or outpatient) or in a nonhospital setting, e.g., a physician directed clinic when the following conditions are met:

- A physician (or a number of physicians) is present to perform medical services and to respond to medical emergencies at all times during patient care hours;
- Each patient is under the care of a physician; and

- All nonphysician services are furnished under the direct, personal supervision of a physician.

Transmittal Information

Transmittal Number

59

Revision History

07/1992 - Provided coverage of apheresis when performed either in an inpatient or outpatient hospital setting or in a nonhospital setting if patient is under the care of a physician and a physician is present to direct and supervise the nonphysician services. Effective date 07/30/1992. (TN 59)

02/1986 - Provided coverage of apheresis for treatment of Guillain-Barre Syndrome and for treatment of life-threatening Systemic Lupus Erythematosus. Effective date 02/14/1986. (TN 4)

Additional Information

Other Versions

Title	Version	Effective Between	
Apheresis (Therapeutic Pheresis)	1	07/30/1992 - N/A	You are here